

# Daily Content Intel

August 21, 2026

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## Topic A — Reel

TELEPROMPTER COPY (paste into Instagram)

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Okay, there's a study out of Wisconsin that changed how I think about testing an athlete before I send them back.

They took 30 athletes who were 1 to 5 years out from ACL reconstruction, plus 30 healthy people, and ran everybody through a single leg vertical hop four different ways. Clean. With a Stroop test running. With stroboscopic glasses that chop up your vision. Then both at once.

Clean hop, people land it about 89 percent of the time. Add the strobe glasses, success drops 17 points. Add the strobe plus the thinking task, it drops 20.

Here's the honest part. The ACL group held up about as well as the healthy group under all that chaos. So be straight about what this is. It's a hard test that trips up healthy people too, and it still needs work before it can sort ready from cleared.

This is what held. 1 to 5 years after surgery, the surgical leg still hopped lower than the other leg. Years later, and it's still there.

So two things Monday. Keep loading that surgical leg long after the paperwork says you're done. And run your testing with the eyes and the brain busy, because that's the only version of the game that actually exists.

Be Good. Help Someone. Learn Lots.

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**Hook:** Put strobe glasses on an athlete mid hop test and the success rate falls 17 points.

**Spoken script (word for word):**

Okay, there's a study out of Wisconsin that changed how I think about testing an athlete before I send them back.

They took 30 athletes who were 1 to 5 years out from ACL reconstruction, plus 30 healthy people, and ran everybody through a single leg vertical hop four different ways. Clean. With a Stroop test running. With stroboscopic glasses that chop up your vision. Then both at once.

Clean hop, people land it about 89 percent of the time. Add the strobe glasses, success drops 17 points. Add the strobe plus the thinking task, it drops 20.

Here's the honest part. The ACL group held up about as well as the healthy group under all that chaos. So be straight about what this is. It's a hard test that trips up healthy people too, and it still needs work before it can sort ready from cleared.

This is what held. 1 to 5 years after surgery, the surgical leg still hopped lower than the other leg. Years later, and it's still there.

So two things Monday. Keep loading that surgical leg long after the paperwork says you're done. And run your testing with the eyes and the brain busy, because that's the only version of the game that actually exists.

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### **On-screen text seeds:**

- 0:00 Strobe glasses. Hop test. Success drops 17 points.
- 0:12 30 post-ACL athletes, 30 healthy. 4 conditions.
- 0:25 Clean 89% success. Strobe 17 points lower. Strobe + Stroop 20 points lower.
- 0:40 Honest read: chaos hurt the healthy group just as much.
- 0:52 1 to 5 years post-op, surgical leg still hops lower.
- 1:05 Keep loading it after clearance. Test with the eyes and brain busy.

### **Caption (copy-paste ready):**

Return-to-sport testing happens in a quiet room. The game does not.

New study in Sports Health: 30 athletes 1 to 5 years out from ACL reconstruction and 30 healthy controls did a single leg vertical hop under 4 conditions. Clean, with a Stroop task, with stroboscopic glasses, and with both.

Clean hop, 89 percent success. Strobe glasses dropped it 17 points. Strobe plus the cognitive task dropped it 20.

The honest finding: the ACL group degraded about the same as the healthy group under load, so treat this as a protocol that reliably produces errors in everybody rather than a screen that sorts ready from cleared. Still useful.

The finding I keep coming back to is the one that held across every condition. 1 to 5 years after surgery, the surgical limb still hopped lower than the other limb. That deficit outlived the clearance letter by years.

Two takeaways. Keep loading the surgical side well past discharge. And test with vision and attention taxed, because that's the environment they're going back into.

It's time to cross your threshold.

Dr. Lars Stevenson, PT, DPT  
threshold.clientsecure.me

#ACLrehab #returntosport #athletictraining  
#physicaltherapy #sportsPT #lacrosse  
#footballtraining #strengthandconditioning  
#kneerehab #ACLrecovery

## **Personalize this**

- You've cleared athletes off hop testing before. Name one where the number said go and your eyes said wait, and what you saw that the test missed.
- How do you currently load the surgical leg in the last phase of ACL rehab, and does anything in your programming keep that side biased after discharge?
- On the field with your football and lacrosse guys, what's the specific visual chaos that breaks down knee control first: a ball in the air, a defender closing, or the turn itself?

## **Shoot notes:**

- Stand against the plainest wall you've got and frame tight from mid-chest up. The

brief's drop-off frames all show empty wall or ceiling filling the top half, so kill the dead space above your head before you hit record.

- Start recording already mid-energy. The first words out are "Put strobe glasses on an athlete mid hop test" and they need to land as a complete thought, not the tail of a sentence you started off-camera.
- Lift the energy a notch through the 17 and 20 point numbers, then drop into a slower, flatter delivery for the "years later" line so the contrast does the work.

**Hook shape:** Statement hook, claim-first, justified by the brief's "Put the payoff in the first line, not after a wind-up" and the 0-8s median 27.5pp drop attributed to a delayed opener payload. Statement sits at 64.5% skip (+0.4pp), acceptable because the first line carries the finding itself rather than setting one up.

## **Screenshots:**

- title |  
<https://pubmed.ncbi.nlm.nih.gov/42605116/>  
| "The Influence of Visual Disruption and

## Dual-Task Conditions on Single-Leg Vertical Hop Performance 1 to 5 Years After Anterior Cruciate Ligament Reconstruction"

- physiology | <https://pubmed.ncbi.nlm.nih.gov/42605116/> | "In the ACLR group, hop height was lower in the surgical limb compared with the nonsurgical limb"
- actionable | <https://pubmed.ncbi.nlm.nih.gov/42605116/> | "The VH protocol was found to be reliable and challenged participants enough to induce errors in both healthy participants and those 1 to 5 years post-ACLR"

### Sources:

- <https://pubmed.ncbi.nlm.nih.gov/42605116/>
- <https://doi.org/10.1177/19417381261471425>



## Topic B — Reel

TELEPROMPTER COPY (paste into Instagram)

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Let's talk about priming day, because I think plenty of programs are running it backwards.

New randomized crossover trial in Sports Health. 10 youth soccer players, 3 days out from a match. One condition was isometric mid thigh pulls, 3 sets of 3 reps, 3 seconds a rep, 10 seconds between reps, 2 minutes between sets. The other condition was their normal training. Retested everybody 24 hours later.

The isometric day did improve theoretical maximal force, and change of direction on the non dominant leg got better, although the authors flag that one as partly a sequencing artifact.

Here's the part I care about. Theoretical maximal velocity went down. Force

orientation at high speed got worse.  
And 30 meter sprint time didn't improve  
in either group.

It's 10 athletes, so hold the study  
loosely. The principle I'll state flat. If  
your athlete's actual gap is that they're  
slow, a priming day that pushes them  
toward force and away from velocity is  
the wrong tool that week. Strength  
without speed is a paperweight.

Prime for force when force is the gap.  
The rest of the time, go sprint.

Be Good. Help Someone. Learn Lots.

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**Hook:** Isometric priming raised these young  
athletes' force and dropped the velocity end  
of their profile 24 hours later.

**Spoken script (word for word):**

Let's talk about priming day, because I think  
plenty of programs are running it backwards.

New randomized crossover trial in Sports  
Health. 10 youth soccer players, 3 days out

from a match. One condition was isometric mid thigh pulls, 3 sets of 3 reps, 3 seconds a rep, 10 seconds between reps, 2 minutes between sets. The other condition was their normal training. Retested everybody 24 hours later.

The isometric day did improve theoretical maximal force, and change of direction on the non dominant leg got better, although the authors flag that one as partly a sequencing artifact.

Here's the part I care about. Theoretical maximal velocity went down. Force orientation at high speed got worse. And 30 meter sprint time didn't improve in either group.

It's 10 athletes, so hold the study loosely. The principle I'll state flat. If your athlete's actual gap is that they're slow, a priming day that pushes them toward force and away from velocity is the wrong tool that week. Strength without speed is a paperweight.

Prime for force when force is the gap. The rest of the time, go sprint.

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### **On-screen text seeds:**

- 0:00 Priming raised force. It dropped top-end velocity.
- 0:10 10 youth soccer players. Randomized crossover. 3 days out from a match.
- 0:22 Isometric mid thigh pull: 3 x 3 reps, 3 seconds, 24 hours before retest.
- 0:38 Force went up. Maximal velocity went down. 30m sprint unchanged.
- 0:52  $n = 10$ . Hold the study loosely.
- 1:02 Prime for force when force is the gap. Otherwise, sprint.

### **Caption (copy-paste ready):**

Priming day costs you something, and most programs never pick which direction they're spending in.

New randomized crossover in Sports Health. 10 youth soccer players, 3 days out from a match, did isometric mid thigh pulls at 3 sets of 3 reps of 3 seconds against their normal training, then retested 24 hours later.

What went up: theoretical maximal force, and change of direction on the non dominant leg (the authors flag that one as sequence-influenced, so I'm holding it lightly).

What went down: theoretical maximal velocity, and force orientation at higher speeds. 30 meter sprint time moved in neither condition.

10 athletes is a small study, so I'd treat the numbers as a signal and hold them loosely. The programming logic holds anyway. Priming shifts an athlete along the force velocity profile, and shifting toward force costs you something at the velocity end. If the reason your kid isn't getting on the field is top speed, that's the exact currency you just spent.

Prime for force when force is the measurable gap. The rest of the time, go sprint.

Dr. Lars Stevenson, PT, DPT  
[threshold.clientsecure.me](mailto:threshold.clientsecure.me)

#speedtraining #strengthandconditioning  
#youthathletes #footballtraining #lacrosse

#forcevelocity #sprinttraining  
#athleticperformance #coaching  
#inseasontraining

## **Personalize this**

- Do you run any kind of priming or potentiation day with your high school football guys, and if so, what's on it and how far out from a game?
- Name an athlete where you called the gap as speed rather than strength. What told you, and what did the block look like after that call?
- Where does isometric work actually earn a slot in your programming right now, and is it there for force output, for tendon, or for a joint angle you can't load dynamically yet?

## **Shoot notes:**

- Different setup than Draft A so the two don't read as one long take. If you shoot the gym, get close enough that the windows and racks fall out of frame; the brief's worst drop-off frame is you in a gym with the background competing for

attention.

- This one is an opinion piece, so bring more voice than Draft A. Flat delivery is named in every single drop-off frame in the brief.
- The "strength without speed is a paperweight" line is your best-performing statement in the library. Slow down and hit it clean, and hold a beat after it before the last two lines.

**Hook shape:** Statement hook, claim-first, contrarian in substance. The brief ranks contrarian shape worst on skip rate (65.5%, +1.4pp), so the counter-consensus content is carried by a statement-shaped opener that states the finding outright rather than teeing up a "everyone thinks X" wind-up.

### **Screenshots:**

- title | <https://pubmed.ncbi.nlm.nih.gov/42584199/> | "Effects of Isometric Priming on the Horizontal Force-Velocity Profile and Multidirectional Speed Performance in Youth Soccer Players"

- physiology |  
<https://pubmed.ncbi.nlm.nih.gov/42584199/>  
| "ISO decreased FV-profile theoretical  
maximal velocity"
- actionable |  
<https://pubmed.ncbi.nlm.nih.gov/42584199/>  
| "ISO priming may represent a practical,  
low-fatigue strategy to enhance selected  
force-oriented performance in youth  
soccer players, although effects on  
velocity-oriented sprint outcomes appear  
limited"

## **Sources:**

- <https://pubmed.ncbi.nlm.nih.gov/42584199/>
- <https://doi.org/10.1177/19417381261469676>



## Reference

### Runner-up topics

- Mike T. Nelson's "sprinkles on the cake" creatine framing (foundation before optimization). Strong hot-take material for the parent audience asking about supplements before their kid sleeps or eats enough. Source: <https://creatine.miketnelson.com> and the Paul Oneid episode.
- Advanced footwear technology improves sprinting mechanics and performance (J Appl Physiol, Aug 14, PMID 42601065). Good "your spikes are doing some of the work" angle for track and field-sport speed testing.
- Lateral jump-landing test with dual-task at return to sport (Phys Ther Sport, Aug 18, PMID 42617458). Same family as Draft A, different source, so it survives dedup as a follow-up piece.

### Sources scanned today

- Newsletter digest  
newsletter\_digest\_2026-08-21.pdf: 1

source (Mercola), pure promo/supplement copy, nothing clinically usable. Fell back to newsletter\_digest\_2026-08-20.pdf, which carried Mike T. Nelson's "Optimal Sprinkles" creatine piece (foundation before optimization). Held as a runner-up.

- PubMed, last 10 days: sprint performance in athletes, hamstring injury prevention, return-to-sport after ACL, youth resistance training, in-season team-sport loading. 40+ titles screened, 3 abstracts pulled in full.
- PubMed, last 30 days: adolescent athlete sleep, creatine in adolescent athletes, low energy availability in adolescent athletes. Nothing athlete-actionable enough to lead with.
- Web scan on dual-task / neurocognitive return-to-sport testing (JOSPT, Translational Sports Medicine pilot, scoping review) corroborates the Draft A theme independently.
- Not reached this run: NSCA blog, BJSM, Pacey Performance and Just Fly show notes, r/AdvancedFitness. PubMed

rate-limited two of five initial queries and they were re-run individually.